Supplementary File 1- PRISMA-P (Preferred Reporting Items for Systematic review and Meta-Analysis Protocols) 2015 checklist: recommended items to address in a systematic review protocol*

Section and topic	Checklist item	Page number
ADMINISTRATIVE INFORMATION		
Title:		
Identification	1a. Identify the report as a protocol of a systematic review	Title page
Update	1b. If the protocol is for an update of a previous systematic review, identify as such	Title page
Registration	2. If registered, provide the name of the registry (such as PROSPERO) and registration number	PROSPERO (CRD42020184055)
Authors:		
Contact	3a. Provide name, institutional affiliation, e-mail address of all protocol authors; provide physical mailing address of corresponding author	Title page
Contributions	3b. Describe contributions of protocol authors and identify the guarantor of the review	11
Amendments	4. If the protocol represents an amendment of a previously completed or published protocol, identify as such and list changes; otherwise, state plan for documenting important protocol amendments	Not applicable
Support:		
Sources	5a. Indicate sources of financial or other support for the review	11
Sponsor	5b. Provide name for the review funder and/or sponsor	11
Role of sponsor or funder	5c. Describe roles of funder(s), sponsor(s), and/or institution(s), if any, in developing the protocol	11
INTRODUCTION		
Rationale	6. Describe the rationale for the review in the context of what is already known	2-3
Objectives	7. Provide an explicit statement of the question(s) the review will address with reference to participants, interventions, comparators, and outcomes (PICO)	4
METHODS		
Eligibility criteria	8. Specify the study characteristics (such as PICO, study design, setting, time frame) and report characteristics (such as years considered, language, publication status) to be used as criteria for eligibility for the review	4-5
Information sources	9. Describe all intended information sources (such as electronic databases, contact with study authors, trial registers or other grey literature sources) with planned dates of coverage	6
Search strategy	10. Present draft of search strategy to be used for at least one electronic database, including planned limits, such that it could be repeated	6; Supplementary file 2
Study records:		
Data management	11a. Describe the mechanism(s) that will be used to manage records and data throughout the review	6-7

Selection process	11b. State the process that will be used for selecting studies (such as two independent reviewers) through each phase of the review (that is, screening, eligibility and inclusion in meta-analysis)	6-7
Data collection process	11c. Describe planned method of extracting data from reports (such as piloting forms, done independently, in duplicate), any processes for obtaining and confirming data from investigators	7
Data items	12. List and define all variables for which data will be sought (such as PICO items, funding sources), any pre-planned data assumptions and simplifications	7
Outcomes and prioritization	13. List and define all outcomes for which data will be sought, including prioritization of main and additional outcomes, with rationale	7
Risk of bias in individual studies	14. Describe anticipated methods for assessing risk of bias of individual studies, including whether this will be done at the outcome or study level, or both; state how this information will be used in data synthesis	8
Data synthesis	15a. Describe criteria under which study data will be quantitatively synthesised	8
	15b. If data are appropriate for quantitative synthesis, describe planned summary measures, methods of handling data and methods of combining data from studies, including any planned exploration of consistency (such as I^2, Kendall's τ)	8
	15c. Describe any proposed additional analyses (such as sensitivity or subgroup analyses, meta-regression)	8
	15d. If quantitative synthesis is not appropriate, describe the type of summary planned	8
Meta-bias(es)	16. Specify any planned assessment of meta-bias(es) (such as publication bias across studies, selective reporting within studies)	8
Confidence in cumulative evidence	17. Describe how the strength of the body of evidence will be assessed (such as GRADE)	10

*** It is strongly recommended that this checklist be read in conjunction with the PRISMA-P Explanation and Elaboration (cite when available) for important clarification on the items. Amendments to a review protocol should be tracked and dated. The copyright for PRISMA-P (including checklist) is held by the PRISMA-P Group and is distributed under a Creative Commons Attribution Licence 4.0.**

From: Shamseer L, Moher D, Clarke M, Ghersi D, Liberati A, Petticrew M, Shekelle P, Stewart L, PRISMA-P Group. Preferred reporting items for systematic review and meta-analysis protocols (PRISMA-P) 2015: elaboration and explanation. BMJ. 2015 Jan 2;349(jan02 1):g7647.

Supplementary File 2- Search Strategy

Search date: 18/7/2020

Database(s): Ovid MEDLINE(R) ALL 1946 - October 2020

Search Strategy:

1	Prejudice/ or Racism/
2	(racism or racial-discriminat* or racial-prejudice or racist-event* or racist-episode* or racial-stereotype* or race-related-stress).tw,kf.
3	((discriminat* or bias* or prejudic* or hostil* or harass* or bully* or cyberbull* or cyber-bull* or (unfair* adj1 treat*) or oppress*) adj3 (race or racial* or ethnic* or cultur* or religio* or migrant* or refugee* or asylum)).tw,kf.
4	(newborn* or new-born* or baby or babies or neonat* or neo-nat* or infan* or toddler* or pre-schooler* or preschooler* or kinder or kinders or kindergarten* or boy or boys or girl or girls or child or children or childhood or pediatric* or paediatric* or adolescen* or youth or youths or teen or teens or teenage* or school-age* or schoolage* or school-child* or schoolchild* or school-girl* or schoolgirl* or school-boy* or schoolboy* or young-person* or young-people).af.
5	Child Welfare/ or pediatric obesity/et, ep, pc
6	(Prejudice/ or *Racism/ or 2 or 3) and 5
7	obesity/et, ep, pc or body mass index/ or overweight/pc
8	Waist-Hip Ratio/
9	Blood Pressure/ or Biomarkers/
10	Hypertension/et, ep, pc
11	exp Cardiovascular Diseases/et, ep, pc
12	depression/et, ep, pc or anxiety/et, ep, pc
13	Mental Health/
14	Stress, Psychological/et, ep, pc
15	Sleep/
16	exp Sleep Wake Disorders/et, ep, pc
17	"Quality of Life"/
18	Resilience, Psychological/ or exp adaptation, psychological/
19	exp substance-related disorders/et, ep, pc or alcohol-related disorders/et, ep, pc
20	smoking/et, ep or exp tobacco smoking/et, ep, pc
21	Mental Disorders/et, ep, pc
22	Self Concept/
23	personal satisfaction/
24	exp suicide/et, ep, pc
25	conduct disorder/et, ep, pc or aggression/et, ep, pc
26	pregnancy outcome/
27	(health-care or healthcare or health-service* or clinic? or ill-health or wellbeing or wellbeing or disease* or illness* or bmi or body-mass-index or anthropometric* or WHR or waist-hip-ratio or hypertension or blood-pressure or cardiometabolic or cardio-metabolic or biomarker* or obese or obesity or overweight or depress* or anxiety or anxious* or mental-health or mental-disorder* or stress or distress* or suicid* or sleep or psychosis or tobacco or smoke* or smoking or drug? or alcohol* or substance-use or substance-related-disorder* or resilien* or self-esteem or self-worth or self-concept or quality-of-life or life-satisfaction or personal-satisfaction or conduct-disorder* or aggression).tw,kf.
28	((social or behavio* or emotion* or developmental* or psychological* or learning*) adj3 (difficul* or problem* or delay* or adjust*)).tw,kf.
29	(((pregnancy or birth or gestation*) and (outcome* or preterm or pre-term or premature or small-for-gestational-age)) or low-birthweight or low-birth-weight).tw,kf.
30	7 or 8 or 9 or 10 or 11 or 12 or 13 or 14 or 15 or 16 or 17 or 18 or 19 or 20 or 21 or 22 or 23 or 24 or 25 or 26
31	*obesity/et, ep, pc or *body mass index/ or *overweight/pc or *Waist-Hip Ratio/ or (*Blood Pressure/ or *Biomarkers/) or *Hypertension/et, ep, pc or exp *Cardiovascular Diseases/et, ep, pc or (*depression/et, ep, pc or *anxiety/et, ep, pc) or *Mental Health/ or *Stress, Psychological/et, ep, pc or *Sleep/ or exp *Sleep Wake Disorders/et, ep, pc or *"Quality of Life"/ or (*Resilience, Psychological/ or exp *adaptation, psychological/) or (exp *substance-related disorders/et, ep, pc or *alcohol-related disorders/et, ep, pc) or (*smoking/et, ep or exp *tobacco smoking/et, ep, pc) or *Mental Disorders/et, ep, pc or *Self Concept/ or *personal satisfaction/ or exp *suicide/et, ep, pc or (*conduct disorder/et, ep, pc or *aggression/et, ep, pc) or *pregnancy outcome/
32	(Prejudice/ or *Racism/ or 2 or 3) and (27 or 28 or 29 or 31) and 4

33	6 or 32
34	limit 33 to (comment or editorial or letter)
35	**33 not 34**

Supplementary File 3- Newcastle - Ottawa Quality Assessment Scale

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE
CASE CONTROL STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Exposure categories. A maximum of two stars can be given for Comparability.

Selection

1) Is the case definition adequate?
   a) yes, with independent validation ✵
   b) yes, eg record linkage or based on self reports
   c) no description

2) Representativeness of the cases
   a) consecutive or obviously representative series of cases ✵
   b) potential for selection biases or not stated

3) Selection of Controls
   a) community controls ✵
   b) hospital controls
   c) no description

4) Definition of Controls
   a) no history of disease (endpoint) ✵
   b) no description of source

Comparability

1) Comparability of cases and controls on the basis of the design or analysis
   a) study controls for _____ (Select the most important factor.) ✵
   b) study controls for any additional factor ✵ (This criteria could be modified to indicate specific control for a second important factor.)

Exposure

1) Ascertainment of exposure
   a) secure record (eg surgical records) ✵
   b) structured interview where blind to case/control status ✵
   c) interview not blinded to case/control status
   d) written self report or medical record only
   e) no description

2) Same method of ascertainment for cases and controls
   a) yes ✵
   b) no

3) Non-Response rate
   a) same rate for both groups ✵
   b) non respondents described
   c) rate different and no designation

NEWCASTLE - OTTAWA QUALITY ASSESSMENT SCALE
COHORT STUDIES

Note: A study can be awarded a maximum of one star for each numbered item within the Selection and Outcome categories. A maximum of two stars can be given for Comparability

Selection

1) Representativeness of the exposed cohort
   a) truly representative of the average _____ (describe) in the community ✵
   b) somewhat representative of the average _____ in the community ✵
   c) selected group of users eg nurses, volunteers
   d) no description of the derivation of the cohort

2) Selection of the non exposed cohort
   a) drawn from the same community as the exposed cohort ✵
   b) drawn from a different source
   c) no description of the derivation of the non exposed cohort

3) Ascertainment of exposure
   a) secure record (eg surgical records) ✵
   b) structured interview ✵
   c) written self report
   d) no description

4) Demonstration that outcome of interest was not present at start of study
   a) yes ✵
   b) no

Comparability

1) Comparability of cohorts on the basis of the design or analysis
   a) study controls for _____ (select the most important factor) ✵
   b) study controls for any additional factor ✵ (This criteria could be modified to indicate specific control for a second important factor.)

Outcome

1) Assessment of outcome
   a) independent blind assessment ✵
   b) record linkage ✵
   c) self report
   d) no description

2) Was follow-up long enough for outcomes to occur
   a) yes (select an adequate follow up period for outcome of interest) ✵
   b) no

3) Adequacy of follow up of cohorts
   a) complete follow up - all subjects accounted for ✵
   b) subjects lost to follow up unlikely to introduce bias - small number lost - > ____ % (select an adequate %) follow up, or description provided of those lost) ✵
   c) follow up rate < ____% (select an adequate %) and no description of those lost
   d) no statement